

RUN DATE:
RUN TIME:
BY:

LABORATORY
Specimen Inquiry

PAGE:1

SPEC #:
STATUS:

Obtained:
Received:

Subm Dr:

CLINICAL HISTORY:
BLADDER CA

IAD-0-3
Carcinoma, urothelial NOS
8/20/13
Site: Bladder NOS
c67.9
9/16/13

SPECIMEN/PROCEDURE:

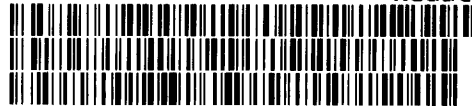
1. LEFT URETER - DISTAL MARGIN
2. RIGHT URETER - DISTAL
3. PROSTATE - AND BLADDER
4. LYMPH NODE, PELVIC - RIGHT
5. LYMPH NODE, PELVIC - LEFT

IMPRESSION:

- 1) LEFT URETER, DISTAL MARGIN, BIOPSY:
 - . Dilated ureter, negative for malignancy.
- 2) RIGHT URETER, DISTAL MARGIN, BIOPSY:
 - . Dilated ureter, negative for malignancy.
- 3) URINARY BLADDER AND PROSTATE, CYSTOPROSTATECTOMY:

UUID:8CDBA39E-6DF0-4EDA-BEF9-086480AE5B42
TCGA-S5-A6DX-01A-PR

Redacted



URINARY BLADDER:

- . Invasive high grade urothelial carcinoma.
 - . Depth of invasion: invading detrusor muscle.
 - . Size of tumor: 2.5 x 1.8 x 1.6 cm.
 - . Lymphovascular tumor emboli present.
 - . Location: posterior inferior bladder wall.
 - . Tumor invades prostate. (see comment)
 - . Resection margins:
 - . Free of tumor.
 - . Tumor is 0.5 cm from the nearest (deep) margin.
 - . Please refer to for prior pathology report.
 - . Please refer to check list for details.
 - . Pathologic TNM (AJCC 7th edition): pT4 N2

pn-TSS, bx 10 days earlier.

PROSTATE:

- . Microscopic foci of prostatic adenocarcinoma, Gleason score 2 + 3 = 5.
 - . Tumor comprises under 0.1 % of the volume of prostate.
 - . Please see check list for details.
 - . Benign stromal hypertrophy.
 - . Pathologic TNM (AJCC 7th edition): pT2a N0
- 4) LYMPH NODE, RIGHT PELVIC, LYMPHADENECTOMY:
 - . No tumor present in 6 lymph nodes. (0/6)

IMPRESSION: (continued)

5) LYMPH NODE, LEFT PELVIC, LYMPHADENECTOMY:

- Metastatic carcinoma consistent with metastatic urothelial carcinoma in 2 out of 5 lymph nodes. (2/5)
- Lymphovascular tumor emboli present.
- Largest metastasis is 0.6 cm in greatest dimension.
- No extracapsular invasion identified.

Dictated by:

Entered:

COMMENT:

Invasion of prostate around the verumontanum is best illustrated in block 3D.

URINARY BLADDER TUMOR CHECKLIST:

SPECIMEN

Bladder and prostate

PROCEDURE

Radical cystoprostatectomy

TUMOR SITE

Posterior inferior wall

TUMOR SIZE

Greatest dimension: 2.5 cm

Additional dimensions: 1.8 x 1.6 cm

HISTOLOGIC TYPE

Urothelial (transitional cell) carcinoma

ASSOCIATED EPITHELIAL LESIONS

None identified

HISTOLOGIC GRADE

Urothelial carcinoma

High-grade

TUMOR CONFIGURATION

Solid/nodule

MICROSCOPIC TUMOR EXTENSION

Tumor invades muscularis propria

Tumor invades deep muscularis propria (outer half)

MARGINS

Margins uninvolved by invasive carcinoma/carcinoma in situ/noninvasive high-grade urothelial carcinoma

Distance of carcinoma from closest margin: 5 mm

Specify margin: deep soft tissue margin

LYMPH-VASCULAR INVASION

Present

COMMENT: (continued)

PRIMARY TUMOR (pT)

pT4: Tumor invades any of the following: prostatic stroma, seminal vesicles, uterus, vagina, pelvic wall, abdominal wall

pT4a: Tumor invades prostatic stroma or uterus or vagina

REGIONAL LYMPH NODES (pN)

pN2: Multiple regional lymph node metastasis in the true pelvis (hypogastric, obturator, external iliac or presacral lymph node metastasis)

Specify: Number of lymph nodes examined: 11

Number cannot be determined (explain):

Number of lymph nodes involved (any size): 2

Location of lymph nodes: Left pelvic

DISTANT METASTASIS

Not applicable

ADDITIONAL PATHOLOGIC FINDINGS

Adenocarcinoma of prostate (use protocol for carcinoma of prostate)

Pathologic TNM (AJCC 7th Edition): pT4 N2

PROSTATE GLAND: RADICAL PROSTATECTOMY CASE SUMMARY

PROCEDURE

Radical cystoprostatectomy

PROSTATE SIZE

Weight: 80 g

Size: 6 x 5 x 4.5 cm

LYMPH NODE SAMPLING

Pelvic lymph node dissection

HISTOLOGIC TYPE

Adenocarcinoma (acinar, not otherwise specified)

HISTOLOGIC GRADE

Gleason Pattern

Primary Pattern

Grade 2

Secondary Pattern

Grade 3

Total Gleason Score: 5

TUMOR QUANTITATION

Proportion (percentage) of prostate involved by tumor: 0.1 %
and/or

Tumor size (dominant nodule, if present):

Greatest dimension: 2 mm

COMMENT: (continued)

EXTRAPROSTATIC EXTENSION

Not identified

SEMINAL VESICLE INVASION (invasion of muscular wall required)

Not identified

MARGINS (select all that apply)

Margins uninvolved by invasive carcinoma

TREATMENT EFFECT ON CARCINOMA

Not identified

LYMPH-VASCULAR INVASION

Not identified

PERINEURAL INVASION

Not identified

PATHOLOGIC STAGING (pTNM)

PRIMARY TUMOR (pT)

pT2: Organ confined

pT2a: Unilateral, involving one-half of 1 side or less

REGIONAL LYMPH NODES (pN)

pN0: No regional lymph node metastasis

DISTANT METASTASIS (pM)

Not applicable

ADDITIONAL PATHOLOGIC FINDINGS

High-grade prostatic intraepithelial neoplasia (PIN)

Benign stromal hypertrophy

ANCILLARY STUDIES

Not performed

Pathologic TNM (AJCC 7th edition): pT2a N0

Entered:

SPECIAL STAINS/PROCEDURES:

Results of immunohistochemistry performed with adequate positive and negative controls are as follow:

Block 3AN:

p63: Basal layer is not preserved in atypical acini.
AMACR: Positive in atypical acini.

Block 3W:

p63: Atypical acini disappeared in deeper section.
AMACR: Atypical acinia disappeared in deeper section.

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SPECIAL STAINS/PROCEDURES: (continued)

IMMUNOHISTOCHEMISTRY DISCLAIMER One or more of the tests in this panel were developed and their performance characteristics determined by the

They have not been cleared or approved by the US Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is regulated under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical testing. Additional information about this assay is available upon request.

Dictated by:

GROSS DESCRIPTION:

1. Received fresh labeled with the patient's name and "left distal ureteral margin" is one piece of tan-red ureter, 0.7 x 0.2 x 0.2 cm. Specimen is entirely submitted for frozen section diagnosis. The remaining frozen tissue is submitted for permanent staining in cassette 1F.

FROZEN SECTION DIAGNOSIS: Segment of ureter, negative for malignancy. Diagnoses rendered by and Dr. nd conveyed to and at

2. Received fresh labeled with the patient's name and "right distal ureteral margin" is one piece of tan-red ureter, 0.5 x 0.3 x 0.3 cm. Specimen is entirely submitted for frozen section diagnosis. The remaining frozen tissue is submitted for permanent staining in cassette 2F.

FROZEN SECTION DIAGNOSIS: Segment of ureter, negative for malignancy. Diagnoses rendered by and and conveyed to and at

3. Received fresh labeled with the patient's name and "bladder, prostate" is one cystectomy prostatectomy specimen, 19.0 x 12.0 x 7.0 cm. The bladder, 5.5 x 4.0 x 4.0 cm, is entirely covered with fibroadipose tissue and is opened with a V incision revealing an ulcerated tumor, 2.5 x 1.8 x 1.6 cm, on the posterior/inferior bladder wall. The remaining bladder wall is pink-tan and grossly unremarkable. The 80 g prostate, 6.0 cm from left to right, 5.0 cm from anterior to posterior, and 4.5 cm from apex to base grossly appears to be involved focally by the bladder tumor at the prostatic bladder base. The remaining prostatic parenchyma is otherwise grossly unremarkable. The bilateral vas deferens and seminal vesicles are grossly unremarkable. The prostate is divided into 8 lamellae and representatively sampled and submitted. The bladder is serially sectioned, representatively sampled, and submitted.

CASSETTE SUMMARY:

CASSETTE 3A:	Tumor with right ureter
CASSETTE 3B-3C:	Tumor with closest deep margin, divided in 2
CASSETTE 3D:	Tumor with bladder base
CASSETTE 3E-3H:	Representative sections of tumor
CASSETTE 3J-3M:	Lamella #1
CASSETTE 3N:	Left vas deferens and seminal vesicle
CASSETTE 3Q:	Right vas deferens and seminal vesicle
CASSETTE 3P:	Prostate apex, serial sections

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GROSS DESCRIPTION: (continued)

CASSETTE 3R: Lamella #3, left anterior
CASSETTE 3S: Lamella #3, left posterior
CASSETTE 3T: Lamella #3, right anterior
CASSETTE 3U: Lamella #3, right posterior
CASSETTE 3V: Lamella #5, left anterior
CASSETTE 3W: Lamella #5, left posterior
CASSETTE 3X: Lamella #5, right anterior
CASSETTE 3Y: Lamella #5, right posterior
CASSETTE 3Z: Lamella #7, left anterior
CASSETTE 3AA: Lamella #7, left posterior
CASSETTE 3AB: Lamella #7, right anterior
CASSETTE 3AC: Lamella #7, right posterior
CASSETTE 3AD-3AX: Remaining unoriented prostate, entirely submitted

4. Received in formalin labeled with the patient's name and right pelvic lymph nodes. Received is a 6.2 x 4 x 2.4 cm aggregate of slightly hemorrhagic yellow gold fibroadipose tissue. The specimen is dissected for possible lymph nodes, 6 possible lymph nodes are identified ranging from 0.9-6 cm in greatest dimension. The specimens is submitted as follows:

CASSETTE 4A: 2 possible lymph nodes
CASSETTE 4B: One lymph node bisected
CASSETTE 4C: One lymph node bisected
CASSETTE 4D: One lymph node representatively submitted
CASSETTE 4E: One lymph node representatively submitted

5. Received in formalin labeled with the patient's name and left pelvic lymph nodes. Received is a 6 x 4.5 x 3.2 cm aggregate of slightly hemorrhagic yellow gold fibroadipose tissue. The specimen is dissected for possible lymph nodes, 5 possible lymph nodes are identified ranging from 1.5-6 cm in greatest dimension. The specimens is submitted as follows:

CASSETTE 5A: One lymph node bisected
CASSETTE 5B: One lymph node bisected
CASSETTE 5C: One lymph node bisected
CASSETTE 5D: One lymph node bisected
CASSETTE 5E-5H: One lymph node representatively submitted

Dictated by:

Entered:

COPIES TO:

Undefined Provider

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CPT Codes:

FS INITIAL/88331/2, PATHOLOGY TISSUE, LYMPH NODE, REGIONAL RESECT/88307/2,
PROSTATE RADICAL RESECTION/88309, URETER BIOPSY-88305/2, BLADDER CYSTECTOMY/88309,
IHC P63-88342, IHC AMACR-88342, NEG MAB CONTROL

ICD9 Codes:

188.9
Resident Physician:

I have personally reviewed the material
(specimen/slide) and approve this final report.

Electronically Signed by: _____
Physicians

** END OF REPORT **

Criteria	<i>bw</i> 5/15/13	Yes	No
Diagnosis Discrepancy			
Primary Tumor Site Discrepancy			☒
HIPAA Discrepancy			☒
Prior Malignancy History			☒
Dual/Synchronous Primary Notes	<i>pract</i>		☒
Case is (circle):	QUALIFIED ☒ / DISQUALIFIED ☐		
Reviewer Initials	<i>BLL</i>	Date Reviewed:	5/9/13